Maplewood General Hospital

400 Orchard Street, New Haven, CT 06511 | Main line (203) 555-0100

Hospital Discharge Summary and Patient Instructions

Patient	Ramesh Mehta	**MRN**	MGH-0048213
Date of birth	03/14/1958	**Unit**	Cardiology 5 East
Admitted	10/12/2026	**Discharged**	10/16/2026
Attending	Ananya Rao, MD (Cardiology)	**Preferred language**	English (daughter also reads Hindi)

Why you were in the hospital

Diagnosis: Acute on chronic heart failure with reduced ejection fraction (HFrEF). Other conditions: high blood pressure, chronic kidney disease stage 3a.

You came in with shortness of breath and leg swelling from extra fluid. We removed about 4 kg (9 lb) of fluid with IV diuretics. Your discharge weight is 78.2 kg (172 lb).

Hospital course (for your care team)

68M with HFrEF (LVEF 30% on TTE 10/13), HTN, CKD 3a who presented with 2 weeks of progressive DOE, orthopnea and bilateral LE edema in the setting of dietary indiscretion. BNP 1,840 on admission. CXR with pulmonary vascular congestion and small bilateral effusions. Diuresed with IV furosemide 80 mg BID with net negative 4.1 L. Cr peaked at 1.6 (baseline 1.3) and improved to 1.4 at discharge. K 4.2. Metoprolol succinate reduced for sinus bradycardia to HR 48. Amlodipine discontinued in favor of GDMT. Spironolactone initiated given LVEF of 30% and preserved potassium. SGLT2 inhibitor deferred to outpatient follow-up pending renal function.

Your medications

START taking these medications

Furosemide (Lasix) 40 mg tablet. Take 1 tablet by mouth once daily in the morning. Purpose: removes extra fluid.

Spironolactone 25 mg tablet. Take 1/2 tablet (12.5 mg) by mouth once daily. Purpose: heart failure; helps the heart and keeps potassium up.

CHANGE how you take these medications

Metoprolol succinate ER 25 mg tablet. Take 1 tablet by mouth once daily. Dose lowered from 50 mg because your heart rate was slow. Do not crush or chew.

CONTINUE taking these medications

Lisinopril 5 mg tablet. Take 1 tablet by mouth once daily. Purpose: blood pressure and heart protection.

Atorvastatin 40 mg tablet. Take 1 tablet by mouth at bedtime. Purpose: cholesterol.

STOP taking these medications

Amlodipine 10 mg tablet. Stop taking. Replaced by your new heart failure medicines.

Ibuprofen (Advil, Motrin) and naproxen (Aleve). Do not take. These can make heart failure and kidney function worse. Use acetaminophen (Tylenol) for pain instead.

Diet and activity

Weigh yourself every morning after you urinate and before breakfast, wearing similar clothing. Write the number down and bring the log to your appointments.

Limit all fluids to 2 liters (about 8 cups) per day. Eat a low-salt diet: less than 2,000 mg of sodium per day.

Walk for 10 to 15 minutes, 2 to 3 times a day. No heavy lifting over 10 pounds for 2 weeks.

Follow-up appointments and tests

Blood test (basic metabolic panel): Tuesday 10/20/2026, any time between 8:00 AM and 4:00 PM, Maplewood Outpatient Lab, Ground floor. No appointment needed. This checks your kidneys and potassium on the new medicines.

Cardiology clinic, Dr. Ananya Rao: Thursday 10/22/2026 at 10:30 AM, Heart and Vascular Center, 2nd floor. Bring your weight log and all your medicine bottles.

Primary care, Dr. James Whitfield: within 2 weeks. Call (203) 555-0142 to schedule.

When to get help

Call the Heart Failure Nurse Line at (203) 555-0187 (24 hours) if: your weight goes up more than 2 pounds in one day or 5 pounds in one week; your legs, ankles or belly swell more; you feel dizzy or light-headed when you stand up; you need more pillows to breathe at night.

Call 911 or go to the nearest emergency room if: you have chest pain or pressure; severe shortness of breath at rest; you faint or pass out; you cough up pink, frothy mucus.